INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Neratinib · extended adjuvant treatment

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00720
Version 2

Daily treatment for one year

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Your record and contact card	14
05	Sources and further information	17

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Reduce recurrence risk as part of the agreed early-breast treatment plan.

ORAL HER-FAMILY KINASE INHIBITOR

Neratinib

Blocks growth signalling inside HER2-positive cancer cells

Enters the cell → HER signalling inhibited → Growth signals reduced

Neratinib binds irreversibly to signalling enzymes in the HER family, including HER2. It is used after trastuzumab-based adjuvant therapy in a specific hormone-receptor-positive, HER2-positive early-breast setting. Its effects on the bowel make early diarrhoea prevention important.

Start the prescribed diarrhoea-prevention plan with the first dose. Report diarrhoea early, especially poor intake, dizziness or reduced urine.

Why this treatment?

Neratinib is an extended adjuvant option for selected hormone-receptor-positive, HER2-positive early breast cancer after trastuzumab-based treatment. Expected benefit, prior HER2 therapy, bowel and liver health, other medicines and the burden of daily treatment guide the discussion.

Your treatment setting

Hormone-receptor-positive, HER2-positive early breast cancer after adjuvant trastuzumab

NCCP 00720 v2 · official regimen

Your exact schedule

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

NCCP 00720 v2 · official regimen

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

NCCP 00720 v2 · official regimen

Why this treatment may be chosen

Discuss your own expected benefit, alternatives and uncertainty with the treating team.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report diarrhoea early. Poor intake, dizziness, reduced urine, blood in stool, fever, severe pain or jaundice needs prompt clinical advice.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00720 v2 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · EMA Nerlynx product information

THE WHOLE TREATMENT JOURNEY

Neratinib · extended adjuvant treatment: treatment and checks

Your treatment pattern

Daily treatment for one year

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First week	Months 1–3	Months 4–12	After one year
Treatment plan NCCP 00720	HR+ / HER2+; <1 year after trastuzumab	Daily tablets + bowel prevention	Continue daily; review tolerability	Complete the planned one-year course	Early-breast follow-up
Blood counts NCCP 00720	Baseline full blood count (FBC)	Review early symptoms	FBC monthly	Monthly to month 6, then every 3 months	Investigate ongoing abnormalities
Liver tests NCCP 00720	Baseline liver profile	Repeat at week 1	Monthly for first 3 months	Then every 6 weeks; sooner if symptoms	Review persistent liver problems
Bowel / heart NCCP 00720	Normal left ventricular ejection fraction (heart pumping measurement) (LVEF) eligibility; loperamide ready	Weeks 1–2 bowel prevention	Weeks 3–8 then PRN bowel plan	Cardiac checks if indicated; preserve prior trastuzumab follow-up	Early-breast follow-up
Cancer review EARLY	Confirm setting	Agree review plan	Follow the early-breast plan	Next treatment / completion	Agreed surveillance

What each check is for

1 **Before treatment**

Before the first dose

Hormone-receptor-positive, HER2-positive early breast cancer after adjuvant trastuzumab. FBC monthly for the first six months, then every three months. Liver tests at week 1, monthly for the first three months, then every six weeks; test earlier for liver symptoms or significant diarrhoea.

Ask / act: Confirm eligibility, test results and medicine list.

2 **Other checks**

At the specified review points

Confirm normal baseline LVEF eligibility; cardiac/LVEF assessment is clinically indicated. Review bowel symptoms, hydration, weight and interacting medicines. Continue any follow-up already required after prior trastuzumab; neratinib does not reset that plan.

Ask / act: Confirm who arranges and reviews each check.

3 **Support and practical plan**

At each treatment or phase change

Prescribed loperamide from the first dose, early hydration and diarrhoea review, liver monitoring and an acid-suppression/interactions plan.

Ask / act: Choose the matching support plan; confirm active medicines.

4 **Response and follow-up**

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.



Blood and organ checks

FBC monthly for the first six months, then every three months. Liver tests at week 1, monthly for the first three months, then every six weeks; test earlier for liver symptoms or significant diarrhoea.

NCCP 00720 v2 · official regimen

Other treatment-specific checks

Confirm normal baseline LVEF eligibility; cardiac/LVEF assessment is clinically indicated. Review bowel symptoms, hydration, weight and interacting medicines. Continue any follow-up already required after prior trastuzumab; neratinib does not reset that plan.

NCCP 00720 v2 · official regimen · EMA Nerlynx product information



Cancer review and follow-up

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

NICE NG101 · bone health and follow-up

The wider treatment plan

Agree surgery, radiotherapy and endocrine treatment as applicable; their schedules and checks remain separate.

Treatment changes

Do not replace a missed dose. Contact the team about diarrhoea or a treatment hold; restart and dose reductions require their instructions.

NCCP 00720 v2 · official regimen



Medicines review

Avoid grapefruit, pomegranate and proton-pump inhibitors. If an H2 blocker is prescribed, take neratinib at least two hours before or ten hours after it; separate antacids by at least three hours. Check CYP3A/P-glycoprotein interactions with the pharmacist.

NCCP 00720 v2 · official regimen

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

One year of tablets, with early bowel and liver support

No dates are entered. Columns show milestones, not equal time intervals; follow your individual prescription.

Treatment or check	Timing in this view
Before starting	**Before starting** Eligibility, tests and medicines review
Treatment	**Weeks 1–2 → Weeks 3–8 → Weeks 9–52** Daily neratinib for one year
Monitoring / support	**Weeks 1–2 → Weeks 3–8 → Weeks 9–52 → Completion** Keep the treatment-specific test and symptom plan
Next decision	**Completion** Confirm continuation or completion

Loperamide prevention starts with the first dose and changes across the early weeks.

Liver tests have an early schedule; do not wait for routine review when symptoms develop.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Diarrhoea prevention

Have the prescribed loperamide plan ready before the first tablet. Call early if diarrhoea starts or worsens.

Hydration

Follow the fluid plan and report dizziness, poor intake or reduced urine.

Liver checks

Keep the week-1 and subsequent liver tests; report yellow skin, dark urine or severe nausea.

Other medicines

Check acid-suppressing medicines and all new prescriptions with the pharmacist.

Taking tablets / attending treatment

Practical treatment details

Make treatment fit into daily life

Take tablets whole with food, preferably in the morning.

Do not crush or dissolve them.

Daily treatment at home

No infusion port or pump is required for neratinib itself.

Questions to ask

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Explain the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Weeks 1–2 · early diarrhoea prevention

Active medicines: Neratinib

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Loperamide	4 mg orally three times daily	Weeks 1–2 from the first neratinib dose NCCP-specific oncology prevention regimen; individual review is required.

Additional phase-specific support

- Start with the first neratinib dose. Titrate under the prescribed plan toward 1–2 bowel movements/day.
- Review constipation, hydration, electrolyte disturbance and diarrhoea severity promptly.
- No generic PPI order: review acid-suppressing medicines and interactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Weeks 3–8 · continuing diarrhoea prevention

Active medicines: Neratinib

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Loperamide	4 mg orally twice daily	Weeks 3–8 Review bowel pattern, hydration and constipation.

Additional phase-specific support

- Titrate under the prescribed plan toward 1–2 bowel movements/day.
- New or worsening diarrhoea may require investigation, fluids and a neratinib hold; do not simply escalate loperamide indefinitely.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Weeks 9–52 · symptom-directed bowel support

Active medicines: Neratinib

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Loperamide	4 mg orally as needed	Weeks 9–52 Prescriber must specify daily maximum and escalation instructions; source gives PRN dosing without a separate maximum here.

Additional phase-specific support

- Review severity, hydration and need to hold neratinib if diarrhoea occurs.
- Rescue antiemetics are PRN and selected after interaction/QT review.
- Confirm acid-suppression timing and the ongoing liver-test schedule.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Neratinib · extended adjuvant treatment

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

Oral HER-family kinase inhibitor · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Bowel / diarrhoea

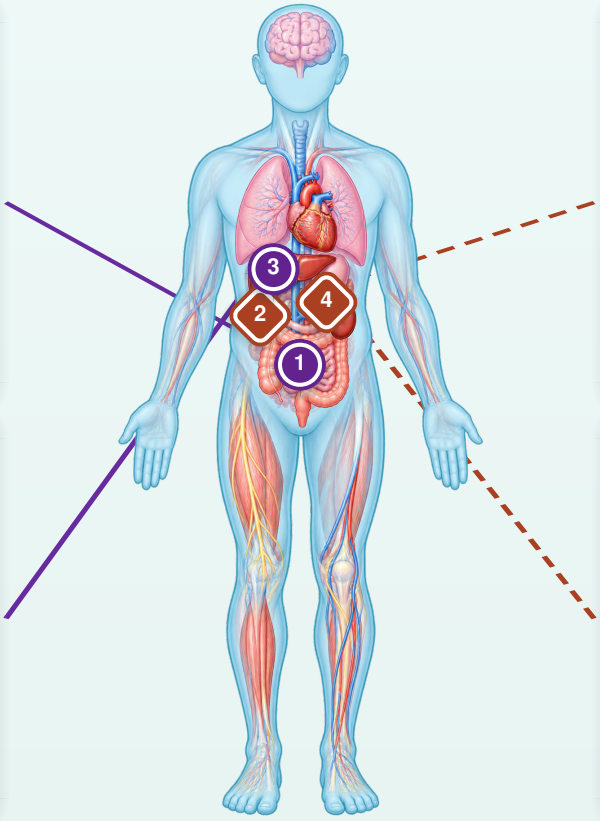
Neratinib

Start the prevention plan; report diarrhoea early.

2 Hydration / kidneys

Neratinib-related fluid loss

Poor intake, dizziness or reduced urine needs prompt advice.



3 Liver

Neratinib

Report jaundice, dark urine or upper abdominal pain.

4 Nausea / appetite

Neratinib

Call if nausea or vomiting prevents food, fluids or tablets.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report diarrhoea early. Poor intake, dizziness, reduced urine, blood in stool, fever, severe pain or jaundice needs prompt clinical advice. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Diarrhoea	□	□	□	
Nausea / vomiting	□	□	□	
Hydration / urine	□	□	□	
Fatigue	□	□	□	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Report diarrhoea early. Poor intake, dizziness, reduced urine, blood in stool, fever, severe pain or jaundice needs prompt clinical advice. Emergency symptoms: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Neratinib · extended adjuvant treatment

NCCP 00720 v2

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Report diarrhoea early. Poor intake, dizziness, reduced urine, blood in stool, fever, severe pain or jaundice needs prompt clinical advice.

https://sactcheck.com/patient/00720/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.19 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00720 v2 · official regimen**
   https://healthservice.hse.ie/documents/6596/720_v2_Neratinib.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **EMA Nerlynx product information**
   https://www.ema.europa.eu/en/documents/product-information/nerlynx-epar-product-information_en.pdf
5. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
6. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
10. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
12. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
13. **NCCP oral antiemetic classification v6 · Tables 5 and 8**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf